Size: 165x120mm

10-01-2023

COLSYN

Colchicine Tablets USP 500 mcg

Composition:
Each uncoated tablet contains:
Colchicine USP 500mcg

Product Description:
White, round, biconvex, uncoated tablets plain on both sides.

Pharmacodynamics:
Pharmacotherapeutic group: drugs for gout, with no effect on uric acid metabolism.

ATC code: M04AC01
The mechanism of action of colchicine in the treatment of gout is not clearly understood. Colchicine is considered to act against the inflammatory response to urate crystals, by possibly inhibiting the migration of granulocytes into the inflamed area. Other properties of colchicine, such as interaction with the microtubules, could also contribute to the operation. Onset of action is approximately 12 hours after oral administration and is maximal after 1 to 2 days.

Pharmacokinetics:
Colchicine is rapidly and almost completely absorbed after oral administration. Maximum plasma concentrations are met usually after 30 to 120 minutes. The terminal half-life is 3 to 10 hours. Plasma protein binding is approximately 30%. Colchicine is partially metabolised in the liver and then in part via the bile. It accumulates in leucocytes. Colchicine is largely excreted (80%) in unchanged form and as metabolites in the faeces. 10-20% is excreted in the urine.
Renal impairment
Colchicine is significantly excreted in urine in healthy subjects. Clearance of colchicine is decreased in patients with impaired renal function. Total body clearance of colchicine was reduced by 75% in patients with end-stage renal disease undergoing dialysis.
Paediatric population
No pharmacokinetics data are available in children.

Indications:
For the treatment of acute gout, and for the prophylaxis of recurrent gout and to prevent acute attacks during initial therapy with allopurinol and uricosuric drugs.

Recommended Dosage:
Treatment of acute gout attack:
1 mg (2 tablets) to start followed by 500 micrograms (1 tablet) after 1 hour.
- No further tablets should be taken for 12 hours.
- After 12 hours, treatment can resume if necessary with a maximum dose of 500 micrograms (1 tablet) every 8 hours until symptoms are relieved.
- The course of treatment should end when symptoms are relieved or when a total of 6 mg (12 tablets) has been taken.
- No more than 6 mg (12 tablets) should be taken as a course of treatment.
- After completion of a course, another course should not be started for at least 3 days (72 hours). *Prophylaxis of gout attack during initiation of therapy with allopurinol and uricosuric drugs:*

500 micrograms twice daily. The treatment duration should be decided after factors such as flare frequency, gout duration and the presence and size of tophi have been assessed.

Route of Administration:
Oral. Tablets should be swallowed whole with a glass of water.
Contraindications:
- Patients with blood dyscrasias
- Pregnancy
- Breastfeeding
- Women of childbearing potential unless using effective contraceptive measures
- Patients with severe renal impairment
- Patients with severe hepatic impairment
- Colchicine should not be used in patients undergoing haemodialysis since it cannot be removed by dialysis or exchange transfusion.
- Colchicine is contraindicated in patients with renal or hepatic impairment who are taking a P-glycoprotein (P-gp) inhibitor or a strong CYP3A4 inhibitor.
- Hypersensitivity to the active substance or to any of the excipients

Pharmacode

Warnings & Precautions:
Colchicine is potentially toxic so it is important not to exceed the dose prescribed by a physician with the necessary knowledge and experience. Colchicine has a narrow therapeutic window. The administration should be discontinued if toxic symptoms such as nausea, vomiting, abdominal pain, diarrhoea occur. Colchicine may cause severe bone marrow depression (agranulocytosis, aplastic anaemia, thrombocytopenia). The change in blood counts may be gradual or very sudden. Aplastic anaemia in particular has a high mortality rate. Periodic checks of the blood picture are essential. If patients develop signs or symptoms that could indicate a blood cell dyscrasia, such as fever, stomatitis, sore throat, prolonged bleeding, bruising or skin disorders, treatment with colchicine should be immediately discontinued and a full haematological investigation should be conducted straight away. Caution is advised in case of:
- liver or renal impairment
- cardiovascular disease
- gastrointestinal disorders
- elderly and debilitated patients
- patients with abnormalities in blood counts

Patients with liver or renal impairment should be carefully monitored for adverse effects of colchicine. Co-administration with P-gp inhibitors and/or moderate or strong CYP3A4 inhibitors will increase the exposure to colchicine, which may lead to colchicine induced toxicity including fatalities. If treatment with a P-gp inhibitor or a moderate or strong CYP3A4 inhibitor is required in patients with normal renal and hepatic function, a reduction in colchicine dosage or interruption of colchicine treatment is recommended.
This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Interactions with other medicaments
Colchicine is a substrate for both CYP3A4 and the transport protein P-gp. In the presence of CYP3A4 or P-gp inhibitors, the concentrations of colchicine in the blood increase. Toxicity, including fatal cases, have been reported during concurrent use of CYP3A4 or P-gp inhibitors such as macrolides (clarithromycin and erythromycin), ciclosporin, ketoconazole, itraconazole, voriconazole, HIV protease inhibitors, calcium channel blockers (verapamil and diltiazem) and disulfiram.
Colchicine is contraindicated in patients with renal or hepatic impairment who are taking a P-gp inhibitor (e.g. ciclosporin, verapamil or quinidine) or a strong CYP3A4 inhibitor (e.g. ritonavir, atazanavir, indinavir, clarithromycin, telithromycin, itraconazole or ketaconazole).
A reduction in colchicine dosage or an interruption of colchicine treatment is recommended in patients with normal renal or hepatic function if treatment with a P-gp inhibitor or strong CYP3A4 inhibitor is required.
A 4-fold reduction in colchicine dosage is recommended when co-administered with a P-gp inhibitor and/or a strong CYP3A4 inhibitor. A 2-fold reduction in colchicine dosage is recommended when co-administered with a moderate CYP3A4 inhibitor.
The magnitude of interactions with strong and moderate CYP3A4 inhibitors as well as with P-gp inhibitors is summarised in the table below:

Single dose of 0.6 mg colchicine without or with:	% change in colchicine pharmacokinetic parameters		Guidance for dose reduction:
	C_{MAX}	$AUC_{o\text{-}t}$	
Strong CYP3A4 inhibitors			4-fold Acute gout regimen to be repeated no earlier than 3 days.
Clarithromycin 250 mg twice daily for 7 days	297	339	
Ketoconazole 200 mg twice daily for 5 days	190	287	
Ritonavir 100 mg twice daily for 5 days	267	345	
Moderate CYP3A4 inhibitors			2-fold Acute gout regimen to be repeated no earlier than 3 days.
Verapamil ER 240 mg once daily for 5 days	130	188	
Diltiazem ER 240 mg once daily for 7 days	129	177	
Grapefruit juice 240 ml twice daily for 4 days	93	95	
Potent P-gp inhibitors			4-fold Acute gout regimen to be repeated no earlier than 3 days.
Cyclosporin 100 mg single dose	324	317	

Given the nature of the side effects, caution is advised with concomitant administration of drugs that can affect the blood count or have a negative effect on hepatic and/or renal function.

In addition, substances such as cimetidine and tolbutamide reduce metabolism of colchicine and thus plasma levels of colchicine increase.

Grapefruit juice may increase plasma levels of colchicine. Grapefruit juice should therefore not be taken together with colchicine.

Reversible malabsorption of cyanocobalamin (vitamin B12) may be induced by an altered function of the intestinal mucosa.

The risk of myopathy and rhabdomyolysis is increased by a combination of colchicine with statins, fibrates, ciclosporin or digoxin.

Potential risk of severe drug interactions, including death, in certain patients treated with colchicine and concomitant P-glycoprotein or strong CYP3A4 inhibitors such as clarithromycin, cyclosporin, erythromycin, calcium channel antagonists (e.g Verapamil and Diltiazem), telithromycin, ketoconazole, itraconazole, HIV protease inhibitors and nefazodone.
P-Glycoprotein or strong CYP3A4 inhibitors are not to be used in patients with renal or hepatic impairment who are taking colchicine.
A dose reduction or interruption of colchicine treatment should be considered in patients with normal renal and hepatic function if treatment with a P-glycoprotein or a strong CYP3A4 inhibitor is required. Avoid consuming grapefruit and grapefruit juice while using colchicine.

Pregnancy:
Fertility
Colchicine administration induces significant reductions in fertility.

Pregnancy
Colchicine is genotoxic, and is teratogenic. Colchicine is therefore contraindicated in pregnancy. Women of childbearing potential have to use effective contraception during treatment.

Breastfeeding
Colchicine is excreted in breast milk. Therefore, use of colchicine is contraindicated in women who are breastfeeding.

Adverse Effects:
The following adverse reactions have been observed. The frequencies are listed under one of the following classifications: Very common, Common, Uncommon, Rare, Very rare and Not known.

Blood and lymphatic system disorders
Not known: bone marrow depression with agranulocytosis, aplastic anaemia and thrombocytopenia.

Nervous system disorders
Not known: peripheral neuritis, neuropathy.
Gastrointestinal system disorders
Common: abdominal pain, nausea, vomiting and diarrhoea.
Not known: gastrointestinal haemorrhage.
Hepatobiliary disorders
Not known: hepatic damage.
Skin and subcutaneous tissue disorders
Not known: alopecia, rash.
Musculoskeletal and connective tissue disorders
Not known: myopathy and rhabdomyolysis.
Renal and urinary disorders
Not known: renal damage.
Reproductive system and breast disorders
Not known: amenorrhoea, dysmenorrhoea, oligospermia, azoospermia.

Pharmacode

Overdose:
Colchicine has a narrow therapeutic window and is extremely toxic in overdose. Patients at particular risk of toxicity are those with renal or hepatic impairment, gastrointestinal or cardiac disease, and patients at extremes of age.
Following colchicine overdose, all patients, even in the absence of early symptoms, should be referred for immediate medical assessment.

Clinical:
Symptoms of acute overdosage may be delayed (3 hours on average): nausea, vomiting, abdominal pain, hemorrhagic gastroenteritis, volume depletion, electrolyte abnormalities, leukocytosis, hypotension in severe cases. The second phase with life threatening complications develops 24 to 72 hours after drug administration: multisystem organ dysfunction, acute renal failure, confusion, coma, ascending peripheral motor and sensory neuropathy, myocardial depression, pancytopenia, dysrhythmias, respiratory failure, consumption coagulopathy. Death is usually a result of respiratory depression and cardiovascular collapse. If the patient survives, recovery may be accompanied by rebound leukocytosis and reversible alopecia starting about one week after the initial ingestion.

Treatment:
No antidote is available.
Elimination of toxins by gastric lavage within one hour of acute poisoning.
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Consider oral activated charcoal in adults who have ingested more than 0.1mg/kg bodyweight within 1 hour of presentation and in children who have ingested any amount within 1 hour of presentation.

Haemodialysis has no efficacy (high apparent distribution volume).

Close clinical and biological monitoring in hospital environment.

Symptomatic and supportive treatment: control of respiration, maintenance of blood pressure and circulation, correction of fluid and electrolytes imbalance.

The lethal dose varies widely (7-65 mg single dose) for adults but is generally about 20 mg.

Effects on Ability to Drive and Use Machine:
No details are available regarding the influence of colchicine on the ability to drive and use machines. However, the possibility of drowsiness and dizziness should be taken into account.

Storage Condition:
Store at temperature not exceeding 30°C. Protect from light

Dosage Forms & Packaging Available:
Uncoated tablet
Alu / PVC blister pack of 10 x 10's

Date of Revision:
January 2023

PL00000-00

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